

Riverside Medical Center

Department of Internal Medicine

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PHYSICIAN PROGRESS NOTE

Patient Name:	SAMPLE PATIENT (De-identified)	MRN:	XXXX-0001
Date of Birth:	01/15/1958 Age: 68	Visit Date:	07/10/2026
Gender:	Male	Attending Physician:	Dr. A. Mitchell, MD
Insurance:	Medicare Part B	Visit Type:	Follow-up / Established Patient
Allergies:	Penicillin (Rash), Sulfa drugs	Code Status:	Full Code

VITAL SIGNS

BP	HR	RR	Temp	SpO2	Weight	Height	BMI
138/86 mmHg	78 bpm	16/min	37.1 °C	97%	84 kg	175 cm	27.4

CHIEF COMPLAINT & HISTORY OF PRESENT ILLNESS

Chief Complaint: Follow-up for Type 2 Diabetes Mellitus and Hypertension; intermittent shortness of breath on exertion for 2 weeks.

HPI: Mr. Sample is a 68-year-old male with T2DM, HTN, and Hyperlipidemia presenting for scheduled follow-up. Home glucose averaging 160–190 mg/dL, up from target range, attributed to dietary non-compliance. Denies chest pain, palpitations, syncope, or lower extremity edema. Notes mild dyspnea climbing one flight of stairs — new since last visit. Denies cough, fever, or recent illness.

PAST MEDICAL / SURGICAL HISTORY

Condition / Procedure	Year	Status
Type 2 Diabetes Mellitus	2014	Active – Metformin + Glipizide
Hypertension	2011	Active – Lisinopril + Amlodipine
Hyperlipidemia	2013	Active – Atorvastatin
Appendectomy	1989	Resolved
Cataract surgery (bilateral)	2022	Resolved

CURRENT MEDICATIONS

Medication	Dose	Route	Frequency
Metformin HCl	1000 mg	PO	Twice daily with meals
Glipizide	5 mg	PO	Once daily before breakfast
Lisinopril	10 mg	PO	Once daily
Amlodipine	5 mg	PO	Once daily
Atorvastatin	40 mg	PO	Once daily at bedtime
Aspirin (low-dose)	81 mg	PO	Once daily

PHYSICAL EXAMINATION

System	Findings
General	Alert and oriented x3. Well-nourished, mild distress with exertion.
HEENT	Normocephalic, atraumatic. PERRL. Oral mucosa moist.
Cardiovascular	RRR. S1/S2 present. No murmurs, rubs, or gallops. No JVD.
Respiratory	Reduced air entry bilateral bases. Mild expiratory wheeze bilaterally. No crackles.
Abdomen	Soft, non-tender, non-distended. Bowel sounds active. No organomegaly.
Extremities	No clubbing, cyanosis, or edema. Peripheral pulses 2+ bilaterally.
Neurological	CN II–XII intact. Sensation intact. Gait normal.
Skin	Warm and dry. No rashes. Mild diabetic dermopathy on bilateral shins.

ASSESSMENT & PLAN

#	Diagnosis / Problem	ICD-10	Plan
1	T2DM — Uncontrolled (HbA1c 8.4%, FBG 186)	E11.65	Increase Glipizide to 10 mg daily. Reinforce diet counseling. Repeat HbA1c in 3 months. Nephrology referral.
2	Hypertension (BP 138/86 mmHg)	I10	Continue Lisinopril 10 mg + Amlodipine 5 mg. Goal BP < 130/80 mmHg. Monitor renal function.
3	Hyperlipidemia (LDL 118, TG 188)	E78.5	Increase Atorvastatin to 80 mg. Target LDL < 70 mg/dL.
4	Dyspnea on exertion — New	R06.00	Order PFTs (Spirometry) and Echocardiogram. Consider cardiology referral per echo result.
5	Mild microcytic anemia (Hgb 12.8, MCV 78)	D64.9	Order serum iron, TIBC, ferritin. Counsel on dietary iron intake.
6	Cholelithiasis (incidental)	K80.20	Asymptomatic — watchful waiting. Return if RUQ pain, fever, or jaundice.

FOLLOW-UP & DISPOSITION

Follow-Up:	3 months (or sooner if symptoms worsen)
Referrals:	Nephrology (diabetic nephropathy); Cardiology (pending echo)
Orders:	PFTs, Echocardiogram, Serum Iron / TIBC / Ferritin
Patient Education:	Diabetic diet, medication compliance, emergency warning signs
Disposition:	Discharged home in stable condition. Patient verbalized understanding.

Electronically Signed by:	Dr. A. Mitchell, MD Internal Medicine	Date/Time:	07/10/2026 09:22 AM
NPI:	1234567890	Provider ID:	RM-PHY-0047